

Patient 7**Report A**

650696224f05/2016 CT Acute Stroke

For Thrombolysis? Initially No, now yes due to crescendo of symptoms

Report:

Time 1st called:05.50 Time plain CT available on PACS: 04.50 Time of verbal report to stroke registrar: 06.05

Indication: Recurrent episodes of left sided weakness and speech disturbance. ?new intracerebral event

CT HEAD

Haemorrhage: No

Intravascular thrombus: No

Acute infarct: NO

Other findings and comments:

CT ANGIOGRAM Window settings: (intraluminal HU=390; W=780; L=290)

(ICA Stenoses NASCET method, measurements on axial 1mm sections Aortic arch and subclavian arteries: Minor predominantly soft scattered atheromatous changes evident in the arch and origins of the great vessels

RCCA/ICA: Minor nonstenotic Soft atheroma at the Carotid bulb

LCCA/ICA. A mixture of calcified and soft, nonstenotic atheroma the carotid bulb

R VA Nondominant. Unremarkable L VA. Dominant. Unremarkable

Intracranial Arteries: No occlusion seen affecting the major basal intracranial arteries. No definite aneurysm or vascular malformation has been demonstrated.

Other findings and comments: Centrilobular emphysematous changes and scarring affects the lung apices bilaterally.

Conclusion: No evidence of an acute cortical or large vessel territorial infarct. No significant findings demonstrated on CTA.

Report B

Clinical information: Left leg weakness. No numbness. No speech disturbance. Discharged yesterday the right MCA lacunar stroke.

Thrombolysed. ? New right MCA stroke.

Findings: There are several punctate foci of diffusion restriction with ADC depression centred upon the right lentiform nucleus in keeping with recent infarcts within the territory of the right middle cerebral artery territory perforators.

There is an additional focus of diffusion restriction centred upon the right caudate head with associated FLAIR hyperintensity but with equivocal ADC depression, an additional subacute infarct with pseudonormalisation is suspected.

A very thin rim of periventricular FLAIR hyperintensity is compatible with minor background chronic small vessel disease. Intracranial time of-flight MRA demonstrates no abnormality, incidental note made of "foetal" right posterior cerebral artery anatomy. These findings were discussed with Stroke team at the time of reporting.